

LEARNER GUIDE

Unit Standard Title: DESCRIBE HOW TO MANAGE REACTIONS ARISING FROM A TRAUMATIC EVENT

Unit Standard No: 244578

Unit Standard Credits: 2

NQF Level: 3

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This material has been compiled based on the following unit standard:

Unit standard 244578: Describe how to manage reactions from a traumatic event

All the above learning material has been accredited by SASSETA in the Republic of South Africa.

Material compiled by:

Training Dynamics Africa

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1. Introduction

This learning program is part of a complete qualification. For more details concerning the complete qualification please contact your service provider or alternatively the office of the SASSETA

2. Purpose of this learning program

This Unit Standard will lead learners to acquire knowledge and skill to effectively manage risks arising from trauma and loss in themselves or others. It could be useful for peer counselors, community counselors and managers to understand the debriefing process and to know to refer individuals who have experienced a traumatic event.

A manager in the context of this Unit Standard is the manager of a small business and first line managers of business units, team leaders and supervisors in medium and large organizations. The term business unit implies a small business, cost centre, section or department.

The Unit Standard was developed as a means of contributing to change in leadership focus and empowering managers to look at people issues.

The qualifying learner is capable of:

- ♦ Recognizing reactions to post trauma stress.
- ♦ Explaining trauma debriefing and the referral process.
- ♦ Explaining possible support strategies.

2.1 Target group

This program is compiled for the following **target group**:

- Security members
- South African Defense Force members
- South African Police Force members
- Correctional Services
- Individuals who wishes to complete the NQF level 3 National Certificate in security practices.

REMEMBER.....

3. Standards and qualifications

It is important that you as the learner understand and realize that this training program is not presented in isolation, but that it was developed based upon nationally recognized standards known as unit standards.

Unit standards are the “building blocks” of qualifications.
All qualifications are plotted on the National Qualifications Framework (NQF).

Unit standards comprises of outcomes. An outcome is a statement that describes the required competency that must be demonstrated by the learner on successful completion of a training intervention.

4. Assessments

In order to assess whether a learner can actually demonstrate the desired outcomes, assessment criteria are included in the unit standard. Each outcome has its own set of assessment criteria.

**REMEMBER.....**

The assessment criteria describe the evidence that is needed that will show that the learner has demonstrated the outcome correctly.

Kindly refer to the unit standard attached hereto for the assessment criteria.

It is of utmost importance that the learner fully understands the assessment criteria as listed in the unit standard, as it is the only way in which the learner will know what he will be assessed against.

The **final or summative assessment** is the most important aspect of this training program. It is during this process that the learner will be declared competent or not yet competent.

The learner will know exactly how he will be assessed, and when and where he will be assessed. All of these details must be obtained from the training provider where the learner enrolled for his program.

5. Range statements

**REMEMBER.....**

Also included in the unit standard are the range statements in support of the assessment criteria.

The range statements indicate detailed requirements of the assessment criteria.

6. The learning guide

The learner guide is included in this material under various learning units. The learner guide has been designed in such a manner that the learner is guided in a logical way through the learning material and requirements of the unit standard.

On completion of this skill program, the learner will be assessed against the assessment criteria as stipulated in the unit standard.

Each of the modules consists of learner outcomes to be achieved during training and development interventions, work related experience or at your own time. The learner outcomes will enable you to achieve the required level of confidence and competence in order to undergo the summative (final evaluations) assessment.

The learner needs to assess his own knowledge and skill throughout the training process by completing the learner workbook. With the completion of the workbook you start collecting evidence to proof your competence. The learner workbook will be assessed (FORMATIVE ASSESSMENT)

TAKE NOTE

The learner guide will remain the property of the learner once the LEARNING PROGRAM has been completed.

TAKE NOTE

- The best results will be obtained if you start with Study Unit 1 in the Learner Guide, and work your way through it, study unit by study unit.
- All learning outcomes are vital and must be studied (and exercised for all practical requirements) thoroughly to ensure that enough evidence for all specific outcomes is generated.

7. Learning assumed to be in place

It is assumed that a learner which registers for this program have completed a program in communication at NQF Level 3 or equivalent.

7.1 Security program matrix

SKILLS PROGRAM 1: SASSETA E				Patrol Security officer/Grade E
244177	Conduct a security patrol in area of responsibility	Level 3	7 Credits	
244184	Apply legal aspects in a security environment	Level 3	8 Credits	
244176	Use security equipment	Level 2	2 Credits	
244179	Handle complaints and problems	Level 3	6 Credits	
244181	Perform hand over and take over responsibilities	Level 3	2 Credits	
244182	Give evidence in court	Level 3	4 Credits	
12484	Perform basic fire fighting	Level 2	4 Credits	
116534	Carry out basic first aid treatment in the workplace	Level 3	2 Credits	
SKILLS PROGRAM 2: SASSETA D				Access control officer/Grade D
244189	Conduct access and egress control	Level 4	7 Credits	
242825	Conduct evacuations and emergency drills	Level 4	4 Credits	
11505	Identify, handle and defuse security related conflict	Level 4	12 Credits	
117705	Demonstrate knowledge of the Firearms Control Act 2000 (Act No. 60 of 2000)	Level 3	3 Credits	
113924	Apply basic business ethics in a work environment	Level 2	2 Credits	
119465	Write/present/sign texts for a range of communicative contexts	Level 3	5 Credits	
114979	Operate a computer workstation in a business environment	Level 3	2 Credits	
SKILLS PROGRAM 3: SASSETA C				Asset & reaction officer/Grade C
113909	Coach a team member in order to enhance individual performance in work environment	Level 3	5 Credits	
13912	Apply knowledge of self and team in order to develop a plan to enhance team performance	Level 3	5 Credits	
244578	Describe how to manage reactions arising from a traumatic event	Level 3	2 Credits	
113852	Apply occupational health, safety and environmental principles	Level 3	10 Credits	
13936	Outline the legal environment of a selected industry	Level 3	2 Credits	
119472	Accommodate audience and context needs in oral/signed communication	Level 3	5 Credits	
11508	Write security reports and take statements.	Level 4	10 Credits	

Unit standard 244578: Describe how to manage reactions from a traumatic event

8. Preparation of summative assessment

After you have worked through the **LEARNER GUIDE** and you are satisfied that all theoretical and practical requirements/ evidence, as stipulated by the UNIT STANDARD, can be met, you need to make your own necessary arrangements with the assessor and/ or facilitator. The training establishment will then arrange a suitable date for you to attend your pre-assessment meeting and then arrange for a suitable date to complete the final assessment.

8.1 RPL assessment



The assessment of RPL learners will be conducted in the same way as for those of new learners. The assessment pack is exactly the same and will therefore be used for new learners as well as RPL Learners. It must however be noted that learners who are applying for RPL must provide proof of previous learning and subject related experience prior to the assessment.

This proof or evidence can be in the format of certified copies (certificates) of previous learning programs that have been attended.

All the evidence will be assessed and authenticated before a learner will be allowed to enroll for an RPL program.

9. Roles and responsibilities

9.1 Facilitator/instructor

- Prepare to facilitate.
- Transfer of knowledge and skills

9.2 The role of the assessor

- Plan for the assessment
- Prepare the learner for the assessment

9.3 The role of the Learner

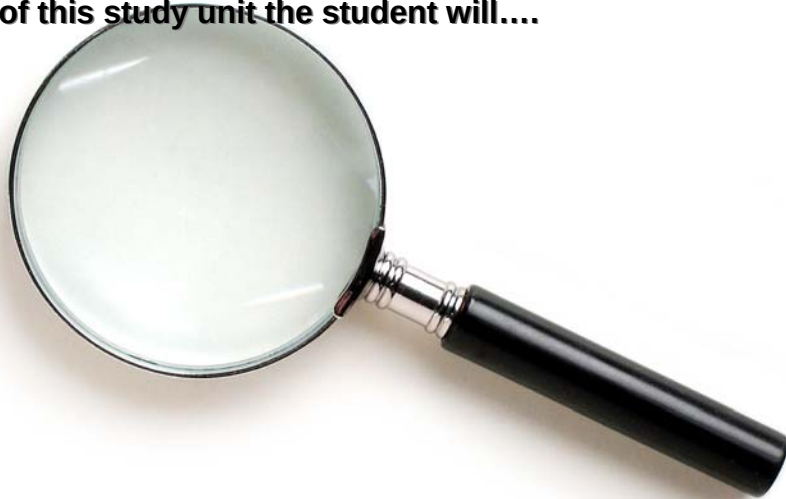
- Prepare for assessment.
- Accept the responsibility of learning.
- Accept the responsibility to produce/deliver evidence.

STUDY UNIT 1

STRESS

LEARNING OUTCOMES

On completion of this study unit the student will....



- ♦ Define the concept trauma by motivating it with examples.
- ♦ Explain the meaning of post traumatic stress.
- ♦ Identify the symptoms of the basic categories of post trauma stress.

1. What is trauma?

DEFINITION

Trauma is an extremely subjective experience. What may be traumatic for one person may barely affect another.

In a general sense though, trauma results when you experience excessive stress that overwhelms your emotional or physical ability to cope.

While emotional trauma can result in the absence of any physical trauma, many times the two go hand-in-hand.

Psychological trauma can last for many years, and if unresolved, can even become more devastating than the original traumatic event.

EXAMPLE:

For instance, the wounds from physical trauma like the loss of a limb or a gunshot wound, while an obvious shock to the body, will eventually heal. What the person is then left with, however, are the emotional wounds and repercussions of the actual traumatic event.

2. What is traumatic stress?

DEFINITION

Post traumatic stress disorder (PTSD) is an anxiety disorder that can develop after exposure to one or more terrifying events in which grave physical harm occurred or was threatened.

It is a severe and ongoing emotional reaction to an extreme psychological trauma.

EXAMPLE:

This stressor may involve someone's actual death or a threat to the patient's or someone else's life, serious physical injury, or threat to physical and/or psychological integrity, to a degree that usual psychological defenses are incapable of coping. In some cases it can also be from profound psychological and emotional trauma, apart from any actual physical harm. Often, however, the two are combined.

3. Symptoms of post-traumatic stress disorder (PTSD)

PTSD develops differently from person to person. If you've lived through a traumatic incident, your symptoms may appear within hours or days of the event, or they may take weeks, months, or even years to develop.

Symptoms can arise suddenly, gradually, or come and go over time. There are three main types of PTSD symptoms:

- ◆ Re-experiencing the traumatic event
- ◆ Avoiding reminders of the trauma
- ◆ Symptoms of hyperarousal or heightened anxiety.

In the days or months following a traumatic event, you may find yourself alternating between re-experiencing the event and avoiding reminders of it, with symptoms of increased arousal as the common backdrop.

3.1 Re-experiencing the traumatic event

The most disruptive symptoms of PTSD involve the.....

- ◆ Flashbacks
- ◆ Nightmares
- ◆ and intrusive memories of the traumatic event.

You may be flooded with horrifying images, sounds, and recollections of what happened. You may even feel like it's happening again. These symptoms are sometimes referred to as intrusions, since they involve memories of the past that intrude on the present.

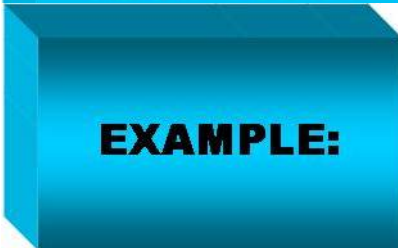
If you have PTSD, you may re-experience the traumatic event or intrusion in several ways:

- ◆ Intrusive memories of the traumatic event
- ◆ Bad dreams about the traumatic event
- ◆ Flashbacks or a sense of reliving the event
- ◆ Feelings of intense distress when reminded of the trauma
- ◆ Physiological stress response to reminders of the event (pounding heart, rapid breathing, nausea, muscle tension, sweating)



TAKE NOTE

These distressing symptoms can appear at any time, sometimes seemingly out of the blue. At other times, they are triggered by something that reminds you of the original traumatic event:



EXAMPLE:

a noise, an image, certain words, a smell.

The person has been exposed to a traumatic event in which **both of the following were present:**

- ♦ The person experienced, witnessed or was confronted with an event or events that involved actual or threatened death or serious injury, or a threat to the physical integrity of self or others.
- ♦ The person's response involved intense fear, helplessness or horror. In children, this may be expressed instead by disorganized or agitated behaviour.

The traumatic event is persistently re-experienced in one or more of the following ways:

- ♦ Recurrent and intrusive distressing recollections of the event, including images, thoughts, or perceptions. In young children, repetitive play may occur in which themes or aspects of the trauma are expressed.
- ♦ Recurrent distressing dreams of the event. In children, there may be frightening dreams without recognizable content.
- ♦ Acting or feeling as if the traumatic event were recurring (includes a sense of reliving the experience, illusions, hallucinations and dissociative flashback episodes, including those that occur on awakening or when intoxicated). In young children, trauma-specific re-enactment may occur.
- ♦ Intense psychological distress at exposure to internal or external cues that symbolize or resemble an aspect of the traumatic event.
- ♦ Physiological reactivity on exposure to internal or external cues that symbolize or resemble an aspect of the traumatic event.

EXAMPLE:**Examples of PTSD Triggers:**

- ♦ **For an auto accident survivor:** The smell of gasoline.
- ♦ **For a combat veteran:** The sound of a helicopter or firecrackers.
- ♦ **For a rape victim:** The sight of a person suddenly appearing around the corner.
- ♦ **For a carjacking victim:** The song that was playing on the radio at the time of the assault.

TAKE NOTE

The intrusions or flashbacks that result from these PTSD triggers are terrifying, disorienting, and unpleasant. The natural response is to protect yourself by avoiding them. This leads to the second major cluster of PTSD symptoms.

3.2 Symptoms of avoidance

Symptoms of avoidance are prominent in PTSD. You may persistently avoid situations that remind you of the traumatic event you experienced, minimize the event's significance, or push all thoughts of it out of your mind. Avoidance can also take the form of detachment and apathy.

EXAMPLE:

Symptoms of avoidance include:

- ◆ Avoiding thoughts, feelings, or conversations associated with the trauma.
- ◆ Avoiding activities, places, or people that remind you of the trauma.
- ◆ Inability to remember important aspects of the trauma.
- ◆ Loss of interest in activities and life in general.
- ◆ Feeling detached or estranged from other people.
- ◆ Feeling emotionally numb, especially toward loved ones.
- ◆ Sense of a limited future (you don't expect to live a normal life span, get married, have a career)

Persistent avoidance of stimuli associated with the trauma and numbing of general responsiveness (not present before the trauma), as indicated by three or more of the following:

- ◆ Efforts to avoid thoughts, feelings or conversations associated with the trauma.
- ◆ Efforts to avoid activities, places or people that arouse recollections of the trauma.
- ◆ Inability to recall an important aspect of the trauma.
- ◆ Markedly diminished interest or participation in significant activities.
- ◆ Feeling of detachment or estrangement from others.
- ◆ Inability to feel certain emotions (**e.g. unable to have loving feelings**).
- ◆ Sense of a foreshortened future (**e.g. does not expect to have a career, marriage, children or a normal life span**).

3.3 Symptoms of increased arousal

PTSD can cause you to feel and react as if you're constantly in danger. In this state of chronic hyperarousal, your mind and body is on constant red alert, making it impossible to fully relax, be productive, or enjoy life.

EXAMPLE:

The PTSD symptoms of increased arousal and anxiety include:

- ◆ Difficulty falling or staying asleep
- ◆ Irritability or outbursts of anger
- ◆ Difficulty concentrating
- ◆ Hypervigilance, or being constantly "on guard"
- ◆ An exaggerated startle response, or jumpiness

Persistent symptoms of increased arousal (not present before the trauma), as indicated by two or more of the following:

- ♦ Difficulty falling or staying asleep
- ♦ Irritability or outbursts of anger
- ♦ Difficulty concentrating
- ♦ Hypervigilance
- ♦ Exaggerated startle responses.

4. The difference between stress and emotional or psychological trauma?

Trauma is stress run amuck. Stress dis-regulates our nervous systems, but for only a relatively short period of time. Within a few days or weeks, our nervous systems calm down and we revert to a normal state of equilibrium.

This return to normalcy is not the case when we have been traumatized. One way to tell the **difference between stress and emotional trauma is by looking at the outcome:**

- ♦ how much residual effect an upsetting event is having on our lives, relationships, and overall functioning.

Traumatic distress can be distinguished from routine stress by assessing the following:

- ♦ How quickly upset is triggered
- ♦ How frequently upset is triggered
- ♦ How intensely threatening the source of upset is
- ♦ How long upset lasts
- ♦ How long it takes to calm down



TAKE NOTE

.....
If we can communicate our distress to people who care about us and can respond adequately, and if we return to a state of equilibrium following a stressful event, we are in the realm of stress.

If we become frozen in a state of active emotional intensity, we are experiencing an emotional trauma, even though sometimes we may not be consciously aware of the level of distress we are experiencing.
.....

5. The symptoms of emotional trauma?

There are common effects or conditions that may occur following a traumatic event. Sometimes these responses can be delayed, for months or even years after the event. Often, people do not even initially associate their symptoms with the precipitating trauma. The following are symptoms that may result from a more commonplace, unresolved trauma, especially if there were earlier, overwhelming life experiences:

5.1 Physical

Adrenaline is a hormone our bodies produce when we are under stress. It "pumps up" the body to prepare it for action. When the stress disappears, the level of adrenaline should go back to normal. In PTSD, it may be that the vivid memories of the trauma keep the levels of adrenaline high. This will make a person tense, irritable, and unable to relax or sleep well.

The hippocampus is a part of the brain that processes memories. High levels of stress hormones, like adrenaline, can stop it from working properly - like "blowing a fuse". This means that flashbacks and nightmares continue because the memories of the trauma can't be processed. If the stress goes away and the adrenaline levels get back to normal, the brain is able to repair the damage itself, like other natural healing processes in the body. The disturbing memories can then be processed and the flashbacks and nightmares will slowly disappear.



- ◆ Eating disturbances (more or less than usual)
- ◆ Sleep disturbances (more or less than usual)
- ◆ Sexual dysfunction
- ◆ Low energy
- ◆ Chronic, unexplained pain

5.2 Emotional

When we are frightened, we remember things very clearly. Although it can be distressing to remember these things, it can help us to understand what happened and, in the long run, help us to survive.

- ◆ The flashbacks, or replays, force us to think about what has happened. We can decide what to do if it happens again. After a while, we learn to think about it without becoming upset.
- ◆ It is tiring and distressing to remember a trauma. Avoidance and numbing keep the number of replays down to a manageable level.
- ◆ Being "on guard" means that we can react quickly if another crisis happens. We sometimes see this happening with survivors of an earthquake, when there may be second or third shocks. It can also give us the energy for the work that's needed after an accident or crisis.

EXAMPLE:

- ◆ Depression, spontaneous crying, despair and hopelessness
- ◆ Anxiety
- ◆ Panic attacks
- ◆ Fearfulness
- ◆ Compulsive and obsessive behaviors
- ◆ Feeling out of control
- ◆ Irritability, angry and resentment
- ◆ Emotional numbness
- ◆ Withdrawal from normal routine and relationships

5.3 Cognitive: Difficulty concentrating on a specific task or experiencing the mind going blank (clouding of consciousness)

EXAMPLE:

- ◆ Memory lapses, especially about the trauma
- ◆ Difficulty making decisions
- ◆ Decreased ability to concentrate
- ◆ Feeling distracted
- ◆ ADHD symptoms

6. The possible effects of emotional trauma

Even when unrecognized, emotional trauma can create lasting difficulties in an individual's life. One way to determine whether an emotional or psychological trauma has occurred, perhaps even early in life before language or conscious awareness were in place, is to look at the kinds of recurring problems one might be experiencing. These can serve as clues to an earlier situation that caused a deregulation in the structure or function of the brain.

6.1 Common personal and behavioral effects of emotional trauma

- ◆ Substance abuse
- ◆ Compulsive behavior patterns
- ◆ Self-destructive and impulsive behavior
- ◆ Uncontrollable reactive thoughts
- ◆ Inability to make healthy professional or lifestyle choices
- ◆ Dissociative symptoms ("splitting off" parts of the self)
- ◆ Feelings of ineffectiveness, shame, despair, hopelessness
- ◆ Feeling permanently damaged
- ◆ A loss of previously sustained beliefs

6.2 Common effects of emotional trauma on interpersonal relationships

- ◆ Inability to maintain close relationships or choose appropriate friends and mates
- ◆ Sexual problems
- ◆ Hostility, arguments with family members, employers or co-workers
- ◆ Social withdrawal
- ◆ Feeling constantly threatened

7. Burn-out

You get burnt out when the continuous stress of dealing with a difficult job or situation becomes too much for you. The most likely candidates for burn-out are hard driven, highly committed people who take on too many tasks or try to do too many things. They suddenly lose interest and motivation for what they're doing.

FACTUAL

You're at risk for burn-out if the following are true for you:

- ♦ You've been trying to achieve too much
- ♦ You are unable to turn down additional work, commitments or responsibilities
- ♦ You've been under intense stress and pressure for a long time
- ♦ You have high standards
- ♦ You don't like to delegate
- ♦ You've been giving others emotional support for a long time
- ♦ You feel guilty when you spend time and money on yourself

7.1 Symptoms of burn-out

Burn-out doesn't happen overnight. It is a process that slowly builds up over a long period of time. Here are some of the most common symptoms of burn-out:

- ♦ Feeling of lack of control over commitments
- ♦ Loss of purpose
- ♦ Loss of motivation
- ♦ Detachment from relationships
- ♦ Feeling tired and lethargic
- ♦ Feeling that you're accomplishing less
- ♦ Increased tendency to think negatively

TAKE NOTE

If you're feeling unmotivated and disillusioned with life, you need help. Contact a good mental health professional and do some of the stress management techniques recommended here. At least, take a break. Life is also about fun and pleasure.

8. Managing stress

Moderate amounts of stress are usually not harmful and can even be stimulating. Excessive stress, however, can be detrimental to one's health and has been linked to immune depression.

A basic stress management plan, based on notes by Pierre Brouard:

- ♦ Know yourself.
- ♦ Find out what stresses you, learn to “read” your symptoms and develop your strengths and work on your weaknesses.
- ♦ Prevent stress. Be alert to the warning signs and listen to your body.
- ♦ Manage your life better. Organise your time, set yourself realistic goals and accept change.
- ♦ at well, eat regular and eat nutritious meals. Don't eat on the run.
- ♦ Exercise moderately and choose something that is also fun to do. Remember that exercise is a natural anti-depressant.
- ♦ Work on your relationships. Learn to be assertive not aggressive, and communicate effectively. Resolve conflict and solve problems constructively.
- ♦ Understand the stress in your work system. Get to know what you can and cannot change and try to see the bigger picture. Accept that conflict is normal in groups.
- ♦ Give yourself permission to be human. Be kind to yourself. Accept yourself. Make peace with what you can and cannot do.
- ♦ Have time out. Do things that are fun. Take time to spoil yourself. Remember sometimes to do nothing.
- ♦ Take care of your soul. Meditate and pray. Nurture your spirit. Stay connected to yourself and to others.

8.1 Look for these hidden signs of stress

There are many other symptoms of stress that are less well known. Here are a few, as well as tips on how to deal with them:

Symptoms	Tip for solution
Duh: Being forgetful and unable to concentrate or make decisions is a symptom of stress. Or maybe you just lack confidence.	Take a stance and stick with it. Better still, take a ten-minute break and a cup of herb tea.
The job's a yawn: If you're yawning a lot and don't feel sleepy, you could be breathing shallowly, causing your body to trigger a yawn to get the oxygen it needs.	Take some deep breaths.

Symptoms	Tip for solution
Blood, sweat and beers: You sweat prolifically, especially under the arms, where the sweat is smellier than elsewhere on your body. The solution? Shower daily, use roll-on and learn to deal with stress. Many people use stimulants such as tobacco, caffeine and particularly alcohol to cope with stress. The problem with nicotine is that it kills you. The problem with booze and caffeine is that it plays havoc with your blood sugar levels, which in turn affects your concentration and moods. Another hormonal result of stress is that you sweat prolifically, especially under the arms, where the sweat is smellier than elsewhere on your body.	The solution: Move to a better neighborhood or learn to recognize and deal with stress quickly. The solution: Shower daily, use roll-on and learn to deal with stress.
Feeling parched: The dry mouth is caused by a decrease in the flow of saliva in the mouth. It can also lead to Death Breath, or halitosis, and dental problems.	Drink water, and floss and brush your teeth regularly.
Sore muscles: When you're stressed, your muscles contract and can end up feeling strained. The back muscles are particularly prone to this, and those in the lower back can go into spasm.	You can ease the muscle pain by doing some squats, resting your arms on your knees. Hold for ten seconds.
Neck strain, sore brain: If you tend to hunch over your keyboard, telephone or steering wheel, your neck muscles will tense up, which can lead to stiffness and eventually headaches. Up to 80 percent of headaches start this way.	Try this: put your hand on your opposite shoulder, feel for tender spots with your fingertips and massage steadily. Take a long, warm bath containing oils such as arnica massage oil, or a few drops of chamomile and lavender oil.
Trouser coughs: Breaking wind constantly might mean you should cut back on the chicken pies and bean soup, but it could also be a sign of stress. Wind, cramps, heartburn and diarrhoea are symptoms of irritable bowel syndrome, which can be triggered by anxiety.	Cut down on caffeine, alcohol and fizzy drinks. Avoid bubble- and chewing-gum, don't drink anything through a straw.
Jumpy legs: Stress can cause an imbalance in the chemicals in your body, which makes your leg muscles jump.	At night, take a couple of magnesium phosphate tissue salt tablets.

Symptoms	Tip for solution
Dandruff attack: Prolonged stress weakens the immune system, making your scalp more vulnerable to the nasty fungi that produce dandruff (scientists examined them under powerful microscopes and found each one to be a tiny snow machine).	Take care of it by always wearing white, or by taking some of the following precautions: towel-dry your hair rather than blow-drying it, which dries the skin; go easy on the anti-dandruff shampoo, which is potent and can dry the scalp, leading to more dandruff; mix some baby shampoo with your regular shampoo a few times each week; or add a few drops of lavender and tea tree oil to your shampoo, or rinse your hair in some lukewarm water containing the oils.
Stuffy and snotty: As we've said, your immune system takes a pounding when you're badly stressed (other symptoms include outbreaks of acne, psoriasis, hives and even warts). This will make you more susceptible to colds and flu.	Arguably the most powerful weapon against this is exercise of the aerobic, weight-bearing sort that lasts at least half an hour, takes place at least three times a week and makes the sweat flow freely. Apart from stimulating the immune system, this has a host of other benefits, such as helping you breathe better and sleep more deeply. Feeling rested will reduce your stress levels.

REMEMBER.....

Exercise helps counteract another symptom of stress weight gain. The stress hormone cortisol helps flab to gather in your mid-section. While exercise has the long-term benefit of keeping you trim, it has one immediate reward: the endorphins it produces make you feel happy, and you needn't even be an athlete with a million-buck sponsorship. It even works for joggers.

(William Smook, Health24, updated July 2008)

[illegible]

STUDY UNIT 2

Trauma debriefing

LEARNING OUTCOMES

On completion of this study unit the student will....



- ♦ Explain debriefing
- ♦ Identify different approaches to be followed when debriefing takes place.
- ♦ Explain the meaning of emotional debriefing.
- ♦ Explain the process of debriefing.
- ♦ Demonstrate knowledge of when debriefing needs to take place
- ♦ Provide contact details for trauma support in your area.
- ♦ Explain a typical process to be followed when a trauma victim needs to be referred.
- ♦ List at least three different debriefing methods.
- ♦ Explain how an employee can be assisted during trauma.

1. Trauma debriefing



Trauma debriefing can be described as early interventions or non-clinical forms of secondary prevention for traumatized people.

2. Different approaches used

♦ Information and advice.



♦ psycho-education),

♦ Self help groups, concrete and direct help



- ♦ Housing
- ♦ Financial assistance
- ♦ Psychological debriefing
- ♦ Crisis intervention
- ♦ Structured trauma counseling and brief psychotherapy

The approach to trauma debriefing currently used by Lifeline is based on Mitchell's 1983 Critical Incident Stress Debriefing (C.I.S.D.) model.

It is used with groups and where appropriate, with individuals who have experienced trauma.

3. Debriefing process

The debriefing process involves the following:

- ♦ Groups of victims of an incident or disaster recounting their impressions and understanding of the event in a systematic and structured form.

The debriefing session is managed and facilitated by a debriefing team or debriefer.

The debriefing process is designed to enable the victim to re-experience the incident in a controlled and safe environment in order to make sense of and become reconciled to the traumatic incident.

4. Emotional debriefing

The availability of early trauma intervention may determine whether the majority of survivors will recover emotionally within a reasonable time or be plagued by delayed symptoms in the future.

Adequate help in the beginning can encourage functional reconstruction of the defenses so that later psychological intervention is less likely to be necessary. Minutes of skilful support and counseling by any sensitive person immediately after a traumatic event can be worth more than hours of professional intervention later.

To this end, 'trauma debriefing' has been promoted as a method, and research has shown a significant decrease in the development of post traumatic stress disorder as a result.

DEFINITION

Emotional debriefing, as a form of trauma treatment, can be defined as a meeting by a counselor with one or more persons, the purpose of which is to review the impressions and reactions that survivors, helpers and others experience during or after a traumatic incident. It is therefore an emotional ventilation of feelings in a controlled and safe environment.

To achieve trauma debriefing, the so-called meta-traumatological treatment model has been developed by H.C. van Wijk, then Manager of the RAU Trauma Centre. This treatment model consists of two main phases, viz. the Emergency Trauma Reduction Phase and the Stabilization phase.

4.1 Phase 1: Emergency Trauma Reduction

The Emergency Trauma Reduction Phase should occur as soon as possible after the traumatic incident, and in any case within 24 hours. It is important to know that no trauma treatment can be administered within the first 24 hours after a traumatic incident, because the victim is still in shock and is not receptive for any counseling. Nevertheless, adequate assistance in the first 24 hours after the incident can provide an important foundation for the functional reconstruction of the victim's ability to immediately commence with the healing process. One of the main effects or symptoms the individual is likely to suffer is the loss of control, and it is therefore very important to start the process of regaining control as soon as possible.

The purpose of the emergency trauma reduction phase is only to support and start the process by assisting the victim to assimilate the trauma experience. The counselor will do the following:

- ♦ Identify major obvious emotions. There may be some immediate expression of anger, fear, blame, sadness or guilt.
- ♦ Identify the personal support system available to the victim, such as a mother, husband, family and friends. It is important to establish contact persons.
- ♦ Facilitate the recovery or safeguarding of personal possessions, if relevant. It may be very upsetting for a person to lose his credit cards and identity documents, for example.
- ♦ Relay immediate further intervention required. **For example**, statements to the police and medical and further trauma treatment may be required.
- ♦ Explain the nature of trauma, and the possible development of symptoms and emotions.
- ♦ Explain the consequences of trauma and the importance of trauma treatment.

4.2 Phase 2: Stabilization

The victim of trauma should preferably be afforded an adequate length of time for the various possible immediate emotional responses, such as denial, disbelief and confusion, to start dissipating. It is for this reason that the stabilization phase cannot be successfully implemented before 24 hours have elapsed.

The victim will then be more ready and able to deal with further emotions such as anger, grief, fear, etc., which may become predominant at this stage, and it is at this juncture that stabilization treatment should commence.

This involves sub-phases of

- ♦ Recounting
- ♦ Releasing
- ♦ Reorientation and
- ♦ Reflection.

The recounting process

During the recounting process the patient recounts what he or she can remember of the period immediately preceding the current specific trauma experienced, the trauma itself and, finally, the period immediately following the current specific trauma experienced. This is important to place the trauma in time and relationship perspective. During the recounting sub-phase the focus is on the objective, rational consideration of the trauma experienced.

The process will typically take two to three recounting. With each session each individual sequence should be expanded in greater detail. New sequences may then emerge which are then inserted into the recounting flow appropriately.

The counselor should intrude or intervene as little as possible during the recounting process, allowing the patient to process the trauma at an increasingly more conscious level.

The counselor must be aware of any indication, by the patient, of the experiencing or reactivation of pre-existing trauma, which may require future revisiting and further treatment.

Releasing process

During the releasing process after the recounting, the patient releases the emotions engendered or evoked as a result of the current specific trauma experienced. The focus is on the subjective or affective consideration of the trauma experienced. The process can reveal a predominant emotion or affect, or a number of them. The emotions may change from one to the other, in no particular order or sequence. It is important to allow the patient to identify and consider the emotions that have been or are being experienced.

The releasing sub-phase will usually require the longest time allocation, as sufficient time must be allowed for the patient to fully deal with the prevalent emotions and for their effect on the patient to be exhausted.

Reorientation

In the reorientation sub-phase the patient begins post trauma reorientation. The counselor plays a somewhat more interventionist role, in that the patient is guided and encouraged into reorientation and returning to a more predictable and structured functioning (living) pattern.

Reorientation commences with the review of where the patient's current trauma position is. The patient does this by recognizing that his or her pre-trauma view of life in terms of concepts, assumptions, perceptions, etc, has been altered by the trauma experienced. This leads to a stage of reflection, when the patient, with the counselor's assistance, reflects on his or her current status compared to prior to the trauma treatment, and how to deal with symptoms such as behavioral and emotional changes in future. The patient should be ready now to be guided into the absolvment or forgiveness process.

A single intervention session is insufficient to ensure effective stabilization, at least six sessions is required for effective intervention. Stabilization should only be implemented as an individual treatment intervention, and not on a group basis. Misapplication or inappropriate misuse of this process can seriously affect the treatment success and future recuperation, and ultimate recovery of the patient. Therefore only counselors who have undergone sufficient and appropriate theoretical and practical education and training, and who are subjected to ethical and professional control, should undertake its provision and application.

5. How do you know if you need a trauma debriefing?

If you have experienced any form of traumatisation, we would recommend that you have a trauma debriefing. It is always better to take preventative measures even if you feel you are coping all right.

Waiting until you discover yourself being overwhelmed by the after-effects of trauma is even more difficult to deal with. Some of the long term consequences of trauma that is not dealt with would be on-going nightmares, anxiety attacks, hypervigilance, flashbacks etc.

A trauma debriefing appears to curtail these after effects, helping people to heal more quickly.

A common response after a traumatic event is the feeling of loss of control over your own life. Through debriefing, the development of adverse reactions to trauma can be prevented by giving you back the perception and experience of control over your life. This control is achieved by enabling you to integrate, at a cognitive and emotional level, the profound personal experiences brought about by the trauma. Where there are a number of people involved in the same incident, the sharing of feelings and information in a group debrief is particularly useful as it enables the individuals to feel less isolated and encourages the acceptance of reactions to the trauma as natural.

The debrief is a way of allowing you to gain mastery of the disaster by actively redefining the experience and its consequences. The mastery experienced enables you to distance yourself from the event. Through this process the traumatic experience can be brought to a conclusion.



REMEMBER.....

Is trauma debriefing different from counseling?

Yes, there is a difference.

Debriefing is not counseling or therapy nor is it a substitute for counseling or therapy. Whilst using some of the basic communication skills used in counseling, it is very different both in content and style from counseling.

Firstly, the debriefing is highly structured with the debriefer guiding you through a series of stages, looking at the traumatic event from different perspectives.

Debriefing usually stands alone as a single psychological crisis intervention and is not part of ongoing therapy. Where deemed necessary or appropriate, follow-up sessions may be suggested by your trauma debriefer.

6. Healing effects of trauma debriefing

- ♦ **It re-establishes hope:** You realize that your feelings are normal and temporary, and that in time, you will become your old self again.
- ♦ **It establishes universality:** It is a normal phenomenon that people are touched by trauma.
- ♦ **It provides ventilation and catharsis:** Group cohesion is healing, there is a common goal of finding a safe harbor.
- ♦ **It provides information:** Which will improve the healing process.

7. Treatment facilities/support services

Counseling at ground level is usually handled by trauma counselors. Psychologists and social workers also administer trauma treatment, but focus more on long-term treatment and not only on trauma. Some institutions, for example churches and police stations, also have trauma units staffed by trained trauma counselors. The Trauma Centre of the Rand Afrikaans University (RAU) developed a National Trauma Network, linking existing trauma centers, psychologist and social workers dealing with trauma.²² The National Trauma Network, again, has developed trauma centers in Eastern Cape, Free State, Gauteng, KwaZulu-Natal, Limpopo and Mpumalanga.

All the trauma counselors in South Africa are part of this National Trauma Network, and whenever there is a specific need the victim can be put in touch with a trauma counselor or trauma clinic. The Network also identifies specific communities where there is a need for a trauma centre, and assists those communities to establish trauma centre and to train trauma counselors according to a standard trauma treatment model.

Apart from properly trained trauma counselors, these centers must have proper facilities that meet the required standards, as well as a manager. A centre's mission and goals should be set, and ways identified by which the goals can be achieved and the resources to do so acquired. The Network provides support to the various centers, and ensure that the centers are on the right track in the attainment of their goals. The aim of control is therefore to check that performance and action confirm to plans to attain the goals and to link with other centers to learn from their experiences or expertise and to support each other.

8. Regional contact detail for support

Western Cape

- ♦ The Trauma Centre 021 465 7373
- ♦ The Trauma Centre For Survivors Of Violence And Torture 021 465 7373
- ♦ Counseling line - +27 (0)21 447 9762
- ♦ Observatory - +27 (0)21 447 1467
- ♦ Counselling line - +27 (0)21 633 9229
- ♦ Manenberg - +27 (0)21 633 5287
- ♦ Counselling line - +27 (0)21 361 908
- ♦ Khayelitsha - +27 (0)21 361 9228 021 661 9085
- ♦ Athlone: 021 633 9229 George: 083 212 6181

KwaZulu Natal

- ♦ Pietermaritzburg: 033 394 4444

Eastern Cape

- ♦ PE [Port Elizabeth]: 041 484 3804

Gauteng

- ♦ JHB 011 642 4345 After hrs :083 765 1235

Life line contact details

- ♦ Lifeline Johannesburg Crisis: (011) 728-1347 Fax: (011) 728-3497
Office: (011) 728-1331
- ♦ Lifeline Western Cape Crisis: (021) 461-1111 Fax: (021) 461-6400
Office: (021) 461-1113
- ♦ Lifeline Durban Crisis: (031) 312 2323 Office: (031) 303 1344 Fax: (031) 303 1419
- ♦ Lifeline Port Elizabeth Crisis: (041) 373-8666 Fax: (041) 373-8880
Office: (041) 373-8882/3
- ♦ Lifeline Free State Crisis: (057) 352-2212 Fax: (057) 352-1065
Office: (057) 357-2746
- ♦ Lifeline Mafikeng Email: llofficel@lantic.net lmaf@lantic.net (Director)
- ♦ National Counseling Line 0861-322-322

9. Organizational referral process and procedures



This is a typical procedure at an organization and can change from organization to organization. It is recommended that the learner find out what does his company's procedures say with regards to trauma debriefing.

The procedure application will include:

- ♦ Traumatic and/or violent events on company premises, such as employees being the victim of or witnessing incidents such as rape, shooting incident, suicide, assault, violent fights, or serious injuries while on duty.
- ♦ Work-related off-company premises events, such as hijacking of staff who are on official business when the event occurs.

The procedure application will exclude:

People who experience trauma in their private capacity. Should such individuals approach the company for help, an appropriate and caring referral process does however need to be followed.

Principles

- ♦ The internal resources will provide first-line support, and refer people to external resources if and when necessary.
- ♦ In the case of a rape or serious sexual assault case, an outsourced offsite service provider can be employed to be the first line of support and post-trauma counseling support. This will be a confidential service with only the organizational health medical practitioner involved in the submission to COIDA. UCT will only receive stats, trends and appropriate risk management information from the service provider.
- ♦ In the case of criminal events, the protocols of SAPS will take precedence over company trauma response to ensure that the opportunity to gather evidence is not lost. The company will however support trauma victims and witnesses whilst they are interviewed by personnel of SAPS.
- ♦ Compliance with statutory and ethical requirements will at all times be maintained.
- ♦ The Personnel department will facilitate and bear the cost of the appropriate professional services to address post-trauma response to any victim/survivor/witness directly involved at the scene of the incident. The extent of this support is outlined in the trauma response procedure.
- ♦ The sensitivity and confidentiality of the case/incident will be maintained at all times without compromising the importance of the health and safety of the greater University community.
- ♦ The Personnel department will conduct an ongoing review of all incidents to establish mechanisms to prevent further such incidents from occurring, and thereby enhancing the safe and fulfilling work environment of this world-class corporate company.

Process:

The process following a work-related violent or traumatic incident is as follows:

- ♦ The victim(s) or the first person to assist at the scene will notify campus protection services.
- ♦ Internal security services will notify a member(s) of the immediate response support team (IRST). Should the event warrant immediate response eg. medical emergency, campus protection services will initiate the action needed.
- ♦ The immediate response support team member(s) offer support, assess the situation, contain the trauma victims and/or witnesses, and refer them for appropriate post-event professional counseling.
- ♦ The immediate response support team member(s) will accompany the person(s) to official bodies such as security services, SAPS (to report a crime), trauma unit or trauma clinic
- ♦ Once all official activities are completed, the IRST member must ensure that the victim is not abandoned. The victim should be accompanied home or to a place where the victim feels safe.
- ♦ The immediate response support team member(s) will keep in contact and provide support to the affected people until the case is closed.
- ♦ For all cases, an incident report (on an annexure 2 form) should be filled in by the H and S representative.
- ♦ Following each incident, a risk analysis will be conducted together with appropriate and reasonable follow-up action, provided by risk management.
- ♦ If appropriate, eg. in the case of an injury or event meeting the COIDA criteria, a COIDA case will be opened.

The COIDA process is:

- ⇒ An employee accident report (WCL2) is to be filled in by the relevant H and S representative and line manager. If this cannot be completed at the time of the event, it may be executed retrospectively and processed as a standard COIDA process.
- ⇒ The victim(s) will be referred to a medical practitioner/trauma unit for assessment and treatment, and a first medical report will be completed by the treating professional practitioner. Should the medical practitioner deem it necessary, the victim(s) will be referred for post-traumatic counselling.
- ⇒ The victim(s) will have to be referred to a post traumatic counselling facility accepted by COIDA (a psychologist or GP trained in post traumatic counselling).
- ♦ If appropriate, eg. no physical injury, the victim will be referred by the IRST member to the trauma clinic and the organisational health medical practitioner will complete the first medical report retrospectively.
- ⇒ On a case to case basis, either the criteria for COIDA acceptance will be applied or the medical advisor of COIDA will be consulted, in order to ascertain whether the case is eligible for submission to COIDA have to be submitted to COIDA.

- ⇒ The victim will have to be referred to a post-traumatic counselling facility accepted by COIDA (a psychologist or GP trained in post traumatic counselling).
- ⇒ Should post traumatic stress disorder be suspected, even up to four weeks post-event, the victim will be referred to a psychiatrist for assessment and confirmation of diagnosis. A report will be forwarded to COIDA. Any management and further treatment may remain with the attending professional service provider, but monthly progress reports will have to be submitted to COIDA.
- ◆ Three post-traumatic counselling sessions with an approved service provider will be paid by the company. The cost will be recouped once the claim has been accepted by COIDA. If the claim is not accepted by COIDA, any other costs incurred will either be covered by the victim's medical aid (when applicable), or for own expense.
- ◆ In the case of a rape or a serious sexual assault, the following alternate process can be used:
 - ⇒ The victim(s) may make immediate contact with the trauma clinic, as this is the approved immediate response support service provider.
 - ⇒ The trauma clinic support response will provide emergency response and containment, which includes accompanying the person(s) to official bodies such as SAPS to report a crime, and to a trauma/rape unit to access appropriate forensic and medical care.
 - ⇒ The trauma clinic will provide the post-traumatic counselling as outlined in this procedure.
 - ⇒ The trauma clinic will initiate the COIDA process (first medical report etc.)
 - * In order to provide a completely confidential and independent service, the COIDA application will be managed by the service provider and the company personnel department health medical practitioner.
 - * On reporting the event to the company personnel department health medical practitioner, the trauma clinic will be issued with a internal security services reference number. This number will be allocated to an event, not to an individual. The company will not be held responsible if this process is not adhered to.
- ⇒ Feedback reports will be provided to the company regarding trends, utilization of services and specific issues or problems. This will be done without revealing the identities of clients who use the service.

Resources Required

Security control

- ◆ Trained staff that understand the implications of traumatic events and who can initiate the trauma response.
- ◆ Management of a Data Base.
- ⇒ Events tracking
- ⇒ Roster of immediate response support team

- ♦ Lay counselors for the immediate response support team.
- ♦ Training resources.
- ♦ Referral network of appropriate professional psychologists and GPs. trained in post-traumatic stress management.
- ♦ Database of support services and places of safety for victims of trauma, for use in their private capacity.

Criteria for specialist service provision

Specialists who support the trauma response team will

- ♦ Be registered with the Health Professions Council of South Africa.
- ♦ Provide 24-hour cover.
- ♦ Be in reasonable proximity to the company premises.
- ♦ Operate throughout the year, including vacation times.
- ♦ Have access to a network that can provide support in case of a mass incident.

10. Debriefing methods

There are a number of therapy techniques that are used. Unfortunately classic PTSD treatment is brief and in no real depth. It is often provided as a response by an employer so that they can claim to have responded and fulfilled their obligations to their employee. This has caused a popularity in short term intervention methods such as "stress debriefing". The method can help, but in many cases the victim of the trauma only finds some help from the process, and has to complete the healing process elsewhere.

10.1 Stress debriefing.

Often called Critical Incident Stress Debriefing. This is the favorite methods of employers to reduce liability claims. It is also preferred by the armed forces since it is designed to get personnel battle ready again in the minimum time possible.

10.2 Counseling:

A longer process that borrows much of its content from grief and bereavement counseling. It recognizes that a number of stages are worked through in order to fully recover from trauma or grief. Some of the stages need time to complete and can not be rushed.

10.3 Psychoanalysis:

A deeper process that looks at why the trauma made such a difference to the individual. This can be effective since some people appear to be traumatized by events that others shrug off. This is due to us all having different formative experiences that combine to make us who we are. Thus different things cause stress for different people. By understanding and treating deeper issues good progress can be achieved.

10.4 Cognitive behavioural therapy:

Life Psycho analysis this looks at why the events effect us in the way they do personally. CBT however tends to be more practical in that there are many exercises and practice things to do, often referred to as "homework" for the client. Many clients like this approach since it gives them an element of control and a good feeling since they are "doing" something tangible to help themselves.

10.5 Deeper trauma debriefing:

The state of hypnosis is useful in deeper debriefing, often referred to as a "Trauma Run". As with typical debriefing, the principle is to "run the trauma" flat of emotion. In a hypnotic state however you are likely to achieve more detail and a more comprehensive result. memory is improved in a state of trance, so the chances of missing any essential details is reduced. The hypnotic state also allows remedial work to be added to the process that is not possible in normal debriefing.

10.6 Disassociated debriefing (NLP).

Some people find the debriefing model too painful since essentially you are reliving events. NLP and Hypnosis allows disassociated recall. In this recall model the person is watching someone else experiencing the trauma, and the pains and emotions are massively reduced. Understanding and information is still forthcoming, but the pain of therapy is less.

10.7 History change (NLP).

A method of therapy was discovered by Richard Bandler, co founder of NLP (Neuro Linguistic Programming). He discovered that it is possible to rewrite areas of memory, replacing them with positive memories and supportive memories. in this therapy you should always ensure full understanding and learning from any memory before tampering with it. As the old motto goes, "he who fails to learn from history is doomed to repeat it". Once you have learnt however, it is possible to play with memory, giving a positive set of emotions. The therapist is of course limited by ethical constraints, and clients considering this option should remember that a good therapist may refuse to change certain memories if they think doing so is unethical or not in the best interests of you, the client.

10.8 Healing environment:

Person Centred Counselling, and holistic therapies both teach us that the body and mind have a good capacity for self healing. For this to occur however the right environment is needed. Therefore resources like a listening ear from a counsellor, regular relaxation therapy and other holistic healing methods are vital if the client is going to self heal.

11. Assisting employees returning to work**11.1 Most common causes of work-related difficulties:**

- ♦ Hypervigilence
- ♦ Site is a trigger and causes an increase in symptoms
- ♦ Secondary injury from employer causing anger and resentment, lack of motivation to return to work
- ♦ Questions and other reminders originating from co-workers, customers, others
- ♦ Memory difficulties, other trauma related symptoms

11.2 Return to work process:

- ♦ Assess employee's desire to return to work
- ♦ Assess manager's willingness to assist in return to work efforts
- ♦ Help employee create a list of activities in increasing intensity of discomfort
- ♦ Ask client to engage in first activity. Limit discomfort to 6/10
- ♦ Assess and continue to increase difficulty of tasks
- ♦ Provide feedback to employer

This image shows a blank sheet of white paper with horizontal ruling lines. The lines are evenly spaced and run across the width of the page. There are no margins, text, or other markings on the paper.

STUDY UNIT 3

Report writing

LEARNING OUTCOMES

On completion of this study unit the student will....



- ♦ Understand and describe the various components of a report.
- ♦ Write a report that will meet all the requirements of a good report.
- ♦ Submit a report for feedback purpose to all the relevant role players.

1. Introduction

Due to the nature of this work, it is often required of a security officer to submit a written report regarding incidents observed. The quality of such a report is of utmost importance as future security actions are often based on the facts contained in the report submitted.

Reporting is a manner in which one person communicates with another. This may either be in writing or verbally. Each method has its advantages and disadvantages.

The written word has the advantage that it can be consulted at a later stage to verify the facts. A disadvantage, however, is that nobody can make use of gestures, tone of voice, etc in the written word, when expressing what they mean.

Care must be taken that the written word (report) conveys to the reader exactly what the writer intends to convey.



A report, in whatever form, can be defined as follows:

A report is a written, systematic account of the actions, observations and findings of the reporter on which he has based his deduction and opinions.

2. Forms of reporting

The reason for the report, as well as the objectives of the writer, will determine the form of the report. A report can adopt any one of the following forms:

- ♦ Covering letters which normally accompany other documents;
- ♦ Letters between various offices on subjects of mutual interest.
- ♦ Circular letters from one office to another, normally to subordinate offices regarding matters of general importance;
- ♦ Requests for one or other privilege, permission, equipment or any other matter directed to higher, parallel or other authority;
- ♦ Reports by a lower authority regarding a matter or incident.

3. Requirements for good reporting

A report must meet certain requirements before it can be classified as a good report. It must be kept in mind that the writer and the reader are separated. The reader cannot question the writer in order to clarify obscurities. It is a waste of time requesting further information by letter. What is more, a poor report causes frustration, both to the reader and writer. The reader is ill-informed and the writer has not achieved his aims.

The following are the general requirements for a good report:

3.1 The principles of an effective report

An effective report must adhere to the following principles:

Accuracy

To be accurate means to be exact and precise. In a report accuracy means that the report is an exact, precise and reliable rendering of the occurrence, condition or situation. All the knowledge the reporter possesses of the subject must be included, without, wittingly or unwittingly, omitting relevant or adding irrelevant particulars.

To ensure accuracy, one must distinguish between each of the following types of information:

- ♦ facts and hearsay;
- ♦ facts and deductions;
- ♦ facts and opinions; and

Having distinguished between the mentioned types of information, does not mean that one or more must be omitted. On the contrary, the reporter must submit **ALL** the information. Should certain information on a report be hearsay, opinions or deductions, the reporter should clearly state this fact for the reader's information.

- ♦ The meaning of words.

REMEMBER.....**Facts**

A fact is something which actually exists and cannot be changed. For a reporter to state the facts concerning any occurrence, condition or situation he must have observed these facts himself by means of one of his five sense organs. The reporter may, however, possess other factual information which was not perceived by him. Such information must also be included in the report, but in such a way that the reader will know that it is not a fact which has been observed by the reporter himself.

REMEMBER.....**Hearsay**

For the purpose of reporting, hearsay is all other information, being fact or fiction, which the reporter did not observe himself. Such information must not be withheld. The reader must, however, know that it is hearsay and that the reporter cannot vouch for the truth or correctness thereof.

REMEMBER.....**Deductions**

A deduction is the conclusion drawn by a person by means of his reasoning. It must be grounded on facts and must be logical. Different persons often make different deductions from the same facts. Enough facts must be submitted to enable the reader to make his own deductions. This must be included in the commentary section of the report and be indicated as such, so that the reader does not confuse the deductions with the facts.

REMEMBER.....**Opinions**

An opinion is a point of view or conviction which a person forms regarding an incident. To carry any weight, an opinion should be based on facts, although this is not essential. The opinion of a reporter is of value to the reader provided the reader is in possession of all the facts. He will then form his own opinion which will often coincide with that of the reporter. The reporter must, however, clearly indicate that it is his opinion in order not to mislead his reader. Opinions are, therefore, usually raised in the commentary part of the report.

REMEMBER.....**The meaning of words**

In the first place the reporter should know the meaning of words before he uses them.

Secondly, it must be kept in mind that one word often has more than one meaning.

The context in which a word is used will determine the significance of the word to the reader. By using a word in the right context the reporter can ensure that he conveys exactly what he intends to convey to the reader.

EXAMPLE:

As an example, we take the case of a person that does not work. We can call him unemployed or a loafer. If he is unemployed, we do not think ill of him, since it is possible that he does not have work due to circumstances. If however, we call him a loafer, immediately there is an unpleasant connotation.

TAKE NOTE

To most people it indicates that this person does not WANT to work although he can find employment. The word "unemployed" causes sympathy whilst "loafer" causes antagonism. Whichever word is to be used will depend on what the reporter wants to convey to his reader.

The reporter must guard against using a word with an ambiguous meaning or making ambiguous statements. Can the reader be blamed if he interprets it incorrectly?

Completeness

A report can only be complete if it contains every fact that is applicable to the event. Accuracy of a report can be established by asking the following questions:

WHO

- ♦ Who committed the offence?
- ♦ Who is the witness?
- ♦ Who is the victim (complainant)?
- ♦ Who reported the case?
- ♦ Who searched the scene of the crime?
- ♦ Who found, marked and dispatched evidence?
- ♦ Who might have had a motive to commit the offence?
- ♦ Who discovered the crime scene?

WHAT

- ♦ What precisely took place? (Describe the events in accurate detail).
- ♦ What object was used to commit the crime?
- ♦ What was done to protect the scene of the crime?
- ♦ What was done to obtain information?
- ♦ What was stolen?

WHERE

- ♦ Provide a complete description of the place, including the name of the building, the street name and number, and the name of the suburb.
- ♦ Where was the complainant?
- ♦ Where was the body of the deceased found?
- ♦ Where were the clues found?
- ♦ Where did the witness stand / sit, etc when the crime took place?

WHEN

- ♦ Note the date and time of the incident.
- ♦ When was it reported?
- ♦ When did the police arrive at the scene?
- ♦ When was the crime discovered?

WHY

- ♦ Describe in detail the events that led up to the incident.
- ♦ Why is a certain person a suspect in the case?
- ♦ Why were certain items stolen, and others not?
- ♦ Why was the deceased murdered?

HOW

- ♦ Describe in detail the actions that constitute the offence.
- ♦ How was the crime committed?
- ♦ How did the person who discovered the crime scene approach the crime scene?

Conciseness

The word "concise" means brief, yet comprehensible, and to the point. In a report "concise", therefore, means that everything that has to be said is said in as few words as possible without omitting essential or relevant particulars.

Objectivity

To be objective means one must not be influenced by one's own emotions or prejudices. Personal feelings and prejudices of the reporter must not be reflected in the report.

The reader can easily be misled by the contents of a report if the reporter's reasoning was dominated by his emotions whilst writing the report.

Comprehensibility

If a report can be read and understood easily by reading it once only, and if the reader can find a specific portion in which he is particularly interested without difficulty, the report will be comprehensive. From the reporter this requires:

- ♦ Good language usage,
- ♦ Proper construction of sentences,
- ♦ Division into paragraphs and sub-paragraphs, and the use of headings and sub-headings where lengthy reports are concerned.

The use of correct punctuation marks plays an important part in avoiding ambiguity. A comma in a wrong place can change the entire context. A missing punctuation mark complicates comprehension. Avoid using long sentences as this makes the report difficult to understand, compels one to read the passage more than once and also causes punctuation errors.

Honesty

To be honest in reporting means to convey the truth without distorting the facts. It has already been said that the reporter must submit **ALL** the information and knowledge concerning the subject matter to the reader. This also means that unfavorable information, either to the reader, the writer or anybody else, may not be withheld. Withholding such information could cause an incorrect decision being made.

4. Guidelines regarding writing a report

Preparation. Before putting pen to paper on a statement form you should have already obtained sufficient detail from your deponent to ensure that you know what you are going to write. Do not forget that wherever you decide to write the statement, it is far better to choose the place where you will have the least interruptions.

5. Composition of a report

A report consists of five parts

- ♦ The heading
- ♦ The body
- ♦ Comments
- ♦ Recommendations
- ♦ Conclusion

The three parts are so interwoven that a definite line cannot be drawn to separate the one from the other.

5.1 The heading

The heading begins on the left hand side of the page after two lines have been skipped below the address of the addressee. It is written in capital letters, is not underlined, and does not end with a full stop.

The heading is a concise summary of the contents of the report. It must contain sufficient information so that no doubt will exist as to the nature of the contents, the subject can easily be identified, it will not be confused with other correspondence, however closely related.

The following requirements are applicable:

- ♦ The heading must contain the subject.
- ♦ The date and place of the occurrence must be mentioned, if applicable.
- ♦ The time of the incident, or the period in which the incident took place, must be stated, if applicable.
- ♦ If any person is involved, he or she must be identified.

The body

One line must be left open below the heading before commencing with the first paragraph.

Paragraphs must be in block form and each paragraph must be numbered in Arabic numerals and a unit, starting with "1". The number must be continuous up to the end of the report. If chapters are used and chapter 1 ends with paragraph 12, chapter 2 will start with paragraph 13.

When it is necessary to subdivide the paragraphs, the following method can be used:

1.
.....
- 1.1
.....
.....
2.
.....
.....

In memoranda, in which different aspects of a matter are dealt with, the division of these aspects into chapters for the use of concise sub-headings is necessary and it is indicated by means of capital Roman numerals, viz Chapter I, II and III.

The contents of a report must be concise and written in the first person. A simple, straightforward and direct style is to be cultivated.

Although exceptions may sometimes be necessary, only one subject is to be dealt with in a particular report.

Reports can sometimes be lengthy. In such instances it is necessary to make use of headings and sub-headings. It is also necessary to give a brief summary of the contents at the beginning of the report, to clearly outline the nature of the contents to the reader. These particulars should, preferably, be given in the first paragraph.

Facts should be recorded in a chronological order, since it is most confusing when the time order of events is mixed up, impairing the requirements of comprehensive-ness.

Annexure and appendixes which are attached to reports must be numbered. Annexure are numbered with capital letters of the alphabet and appendixes with Arabic numerals, i.e. Annexure "A", "B" and Appendix 1 and 2.

**The difference between an annexure and an appendix is.....**

An annexure is a document without which the report would be incomplete, for example Annexure to an indictment.

An appendix is an explanatory document with which the original writing would, in any case, be complete viz a map of the surroundings.

Comment

Commentary is sometimes superfluous but often necessary. After reading the contents of the report, the reader may raise the following questions:

- ♦ How authentic and reliable is the informer as far as hearsay facts are concerned?
- ♦ What is the background of the person involved?
- ♦ Is there any connection between the present case and a previous incident?
- ♦ Can anyone else cast any light on the subject?
- ♦ Is there any other information which could not be included in the report, but could help to clarify the situation?

The reporter was on the scene and investigated the matter. It is just logical that he will possess information that may be relevant but cannot be stated as a fact. It is only fair to supply the reader with all the information, since he often has to make a decision which might have far-reaching consequences to himself or someone else. This additional information may be conveyed under the heading "COMMENT".

It is necessary that the heading "COMMENT" be used. To the reader it is immediately clear that the reporter has given all the factual evidence he possesses.

This part of the report is also the most suitable place for the writer to give his opinion or to draw attention to his own deductions.

At the same time, the reporter can mention the steps he has taken pending instructions from his superior.

The "Comment" part forms one paragraph only, with different sub-paragraphs if necessary.

Recommendations

The purpose of a recommendation is to convince the reader to take a decision which the reporter deems the most suitable under the prevailing circumstances.

In almost every report it is necessary for the reporter to make a recommendation, even if it was not requested for by the authority ordering the report to be submitted.

When a recommendation is made, care must be taken not to use words which could be interpreted as an order or instruction.

It is useless to submit an unmotivated recommendation. Such a recommendation is meaningless and carries no weight. Should the writer recommend a specific action without indicating why he has considered it as the most suitable solution, he has missed his goal.

The conclusion

The report is concluded with the signature of the writer on the left-hand side of the page.

Due to signatures often being illegible, the initials and surnames of the signatory must be typed.

Should the writer fill a designated post, it must be indicated in capital letters below his initials and surnames.

6. Feedback

The report are used to provide feedback to the following key role players.

- ◆ Management
- ◆ The client
- ◆ Security personnel
- ◆ Emergency services



Please take note: It is extremely important to provide feedback on all the facts and methods that have been used to defuse /handle the conflict situation.

The situation can also be recorded in an incident report or the site Occurrence book.

[illegible]

WORDS/ACRONYMS	EXPLANATION OF ACRONYMS
ASSESSMENT	"A structured process of gathering evidence and making judgments about an individual's performance in relation to registered national standards and qualifications"
ASSESSMENT CRITERIA	It describe the evidence that is needed to show that the learner has demonstrated the outcome correctly. A list of evidence to be supplied by the candidate as proof of his / her ability to integrate all of the competencies listed earlier in the standard.
COMPETENCE	Competence is the ability to do certain things in a range of different environments with knowledge and understanding about what is being done and the effect this has on others. It includes <u>Knowledge, skills, attitudes, values and ethics.</u>
CREDIT	Each unit standard is worth a number of credits which gives an indication of the size and scope of the unit standard, the standard is worth. (1 credit = 10 notional hours of learning)
CRITICAL CROSSFIELD OUTCOMES	critical cross field outcomes is to direct educational activities towards development of the learners within a social and economic environment.
EMBEDDED KNOWLEDGE	Body of theory essential for understanding and applying the standard. Principles, methods and techniques that would underpin good practice of this unit standard.
EVIDENCE	Evidence is a set of information or observations that provides proof of the learner's competence.
FORMATIVE ASSESSMENT	Aims to assist the learner to identify any knowledge/skills gaps when demonstrating his/her current competence. Diagnostic assessment supports learning and progress of the learner.
INTEGRATED ASSESSMENT	Integrated assessment refers to: • assessing a number of outcomes together • assessing a number of assessment criteria together • assessing a number of unit standards together • using a combination of assessment methods and instruments for an outcome/s • collecting naturally occurring evidence in the workplace • Acquiring evidence from other sources such as supervisor's reports, testimonials, portfolio of work previously done, logbooks, journals etc.
LEARNING ASSUMED TO BE IN PLACE	Building blocks (knowledge, skill and understanding) assumed to be in place
LEARNING PROGRAM	A "learning program is a set of learning activities in which the learner will be involved in working towards the achievement of one or more outcomes.
LEVEL	Each unit standard is registered on the NQF at a particular level based on the complexity of the outcomes and stringency of the standards.
NQF	National Qualifications Framework
RANGE STATEMENTS	The range statements indicate detailed requirements of the assessment criteria.

RE-ASSESSMENT	• The same method and instrument may be used, but, the task and materials should be different • The task and materials should, however, be of the same complexity and level as the previous ones. • In case the methods and instruments are changed it must be ensured that they are appropriate for the outcomes specified.
RPL ASSESSMENT	RPL is a way of recognizing what individuals already know and can do. The basic premise underlying it is that people, especially mature adults, learn many things inside and outside the formal structures of education and training and that irrespective of where, how and when learning was acquired, subject to assessment, it can be worthy of recognition and credit.
RPL	Recognition of prior learning
SASSETA	Safety & security sector education and training authority.
SAQA	SOUTH AFRICAN QUALIFICATION AUTHORITY
SPECIFIC OUTCOMES	An outcome is a statement that describes the required competency that must be demonstrated by the learner on successful completion of a training intervention. Key performance areas in the standard are given, the focus is <i>not</i> only on current competence, but also on future competence in this standard.
SUMMATIVE ASSESSMENT	Assessment used to record the level of competence achieved by a learner over a period of time, e.g. at the end of a program, term or year.
QUALIFICATIONS	Represent a planned combination of learning outcomes which has a defined purpose and intended to provide learners with applied competence and a basis for further learning
UNIT STANDARDS	the “building blocks” of qualifications. All qualifications are plotted on the National Qualifications Framework (NQF). “A statement of the outcomes that the learner must achieve to earn credit. Standards are nationally agreed and internationally comparable.”

Describe how to manage reactions arising from a traumatic event

SAQA US ID		UNIT STANDARD TITLE	
114942		Describe how to manage reactions arising from a traumatic event	
ORIGINATOR		REGISTERING PROVIDER	
SGB Life Skills			
FIELD		SUBFIELD	
Field 07 - Human and Social Studies		People/Human-Centred Development	
ABET BAND	UNIT STANDARD TYPE	NQF LEVEL	CREDITS
Undefined	Regular-Fundamental	Level 3	2
REGISTRATION STATUS	REGISTRATION START DATE	REGISTRATION END DATE	SAQA DECISION NUMBER
Passed the End Date - Status was "Reregistered"	2007-06-26	2007-08-16	SAQA 0160/05
LAST DATE FOR ENROLMENT		LAST DATE FOR ACHIEVEMENT	
2008-08-16		2011-08-16	

PURPOSE OF THE UNIT STANDARD

This Unit Standard will lead learners to acquire knowledge and skill to effectively manage risks arising from trauma and loss in themselves or others. It could be useful for peer counsellors, community counsellors and managers to understand the debriefing process and to know to refer individuals who have experienced a traumatic event.

A manager in the context of this Unit Standard is the manager of a small business and first line managers of business units, team leaders and supervisors in medium and large organisations. The term business unit implies a small business, cost centre, section or department.

The Unit Standard was developed as a means of contributing to change in leadership focus and empowering managers to look at people issues.

The qualifying learner is capable of:

- Recognising reactions to post trauma stress.
- Explaining trauma debriefing and the referral process.
- Explaining possible support strategies.

LEARNING ASSUMED TO BE IN PLACE AND RECOGNITION OF PRIOR LEARNING

It is assumed that learners are competent in Communication at NQF Level 1.

UNIT STANDARD RANGE

The typical scope of this Unit Standard is:

- A traumatic event includes, but is not limited to, loss or trauma resulting from crime and violence, terrorism, sexual assault, road accidents, death, divorce, AIDS, serious terminal illness and natural disasters.

UNIT STANDARD OUTCOME HEADER

N/A

Specific Outcomes and Assessment Criteria:**SPECIFIC OUTCOME 1**

Recognise reactions to post trauma stress.

ASSESSMENT CRITERIA**ASSESSMENT CRITERION 1**

The concept of trauma is explained with examples.

ASSESSMENT CRITERION 2

The concept of post trauma stress is explained with examples.

ASSESSMENT CRITERION 3

Observable cognitive symptoms that could indicate post trauma stress are described with examples.

ASSESSMENT CRITERION RANGE

Includes, but is not limited to; decision making ability, concentration, memory and disorientation.

ASSESSMENT CRITERION 4

Observable emotional symptoms that could indicate post trauma stress are described with examples.

ASSESSMENT CRITERION RANGE

Includes, but is not limited to, loss of emotional control, inappropriate emotional response, anxiety and depression.

ASSESSMENT CRITERION 5

Observable behavioural symptoms that could indicate post trauma stress are described with examples.

ASSESSMENT CRITERION RANGE

Includes, but is not limited to, behavioural changes, anti social acts, substance abuse.

ASSESSMENT CRITERION 6

Observable physical symptoms that could indicate post trauma stress are described with examples.

ASSESSMENT CRITERION RANGE

Includes, but is not limited to, respiratory problems, fatigue and illness.

SPECIFIC OUTCOME 2

Explain trauma debriefing and the referral process.

ASSESSMENT CRITERIA**ASSESSMENT CRITERION 1**

The concept of trauma debriefing is explained and an indication is given of when debriefing is appropriate.

ASSESSMENT CRITERION 2

The process and techniques followed in trauma debriefing are outlined with examples.

ASSESSMENT CRITERION 3

Support services available for trauma debriefing are identified in a specific area.

ASSESSMENT CRITERION 4

The process to be followed in referring an individual for debriefing is explained with reference to the process of a specific organisation.

SPECIFIC OUTCOME 3

Explain possible support strategies.

ASSESSMENT CRITERIA**ASSESSMENT CRITERION 1**

Possible post trauma support strategies are explained and an indication is given of when each is appropriate.

ASSESSMENT CRITERION 2

Ways in which a traumatised individual can be reasonably accommodated are identified for a specific situation.

ASSESSMENT CRITERION 3

A report on how the individual is to be reasonably accommodated in a specific situation is compiled and communicated to the appropriate parties.

UNIT STANDARD ACCREDITATION AND MODERATION OPTIONS

This Unit Standard will be internally assessed by the provider and moderated by a moderator registered by a relevant accredited ETQA or an ETQA that has a Memorandum of Understanding with the relevant accredited ETQA.

UNIT STANDARD ESSENTIAL EMBEDDED KNOWLEDGE

N/A

UNIT STANDARD DEVELOPMENTAL OUTCOME

N/A

UNIT STANDARD LINKAGES

N/A

Critical Cross-field Outcomes (CCFO):**UNIT STANDARD CCFO COLLECTING**

The learner is able to collect, organise and evaluate information in recognising the symptoms of post-traumatic stress.

UNIT STANDARD CCFO COMMUNICATING

The learner is able to communicate effectively in compiling a report on the reasonable accommodation of an individual in a specific situation.

UNIT STANDARD CCFO DEMONSTRATING

The learner is able to demonstrate cultural sensitivity in referring an individual and proposing a strategy to accommodate an individual in a specific situation.

UNIT STANDARD ASSESSOR CRITERIA

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- ♦ http://en.wikipedia.org/wiki/Post-traumatic_stress_disorder
 - ♦ [/bizlinks.wordpress.com/2008/05/29/contact-numbers-for-trauma-debriefing-counseling/](http://bizlinks.wordpress.com/2008/05/29/contact-numbers-for-trauma-debriefing-counseling/)
 - ♦ <http://www.lifelinewc.org.za/trauma.htm>
 - ♦ <http://www.trauma-pages.com/support.php>